



CCMA Certificates & Full Narrative Evidence Bundle – Wilbur Matthee

5 messages

Wilbur William Matthee <wilburmat@gmail.com>
To: Marco@schroterlaw.co.za
Bcc: wilburmat@gmail.com

Fri, 20 Feb 2026 at 10:09

Dear Marco,

Following our telephone conversation, please find the attached CCMA Certificates of Outcome as requested.

In addition to the formal V11 Statutory Affidavit I sent earlier, I am also providing a Narrative-Based Version of the affidavit. This version is more detailed and contains active links to my Google Drive, which houses the full evidence bundle and all corresponding annexures.

Security Note: As we discussed regarding the "vanishing" emails and the compromise of my digital integrity, please be aware that I am monitoring access to these Drive folders closely. If you have any trouble accessing the links or if you prefer I send the files via a different secure method, please let me know immediately.

Please acknowledge receipt of this email and the attachments, as I am relying on your office to help secure the "paper trail" that is currently being tampered with on my end.

Kind regards,

Wilbur Matthee
Specialist Nurse
PG Dip Health Services Management
BCur Health Nursing Management
BTech General Nursing (Psychiatry, Midwifery & Community Health)

Leadership Roles: Acting Chair – Employee Health & Safety Committee
Member – Postgraduate Student Quality Committee

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2 attachments

 **K1: Certificate of Non-Resolution (Outcome).jpg**
1.3 MB

 **V_32 Founding Affidavit.PDF**
867 KB

Marco van der Walt <marco@schroterlaw.co.za>
To: Wilbur William Matthee <wilburmat@gmail.com>

Fri, 20 Feb 2026 at 10:40

Hi Wilbur,

I confirm receipt.

Yours faithfully,



MARCO VAN DER WALT

Senior Associate & Conveyancer

BComm LLB (NWU)

[132 Merriman Street, George Central, George, 6529](#)

Tel: (044) 874 5372

marco@schroterlaw.co.za | www.schroterlaw.co.za

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Marco van der Walt <marco@schroterlaw.co.za>
To: Wilbur William Matthee <wilburmat@gmail.com>

Thu, 26 Feb 2026 at 11:43

Dear Wilbur,

I trust you are doing well.

I confirm that I have considered the abundance of documentation provided.

Kindly advise (in short):

1. The date of your protected disclosure;
2. To whom it was made; and
3. What was the exact subject matter thereof.

[Quoted text hidden]

[Quoted text hidden]

Dear Marco,

Wilbur William Matthee <wilburmat@gmail.com>
To: Marco van der Walt <marco@schroterlaw.co.za>

Thu, 26 Feb 2026 at 13:08

Dear Marco,

Thank you for engaging with the materials.

The short answer to your three questions:

1. DATE OF PROTECTED DISCLOSURE:

There are three key protected disclosures forming an escalating pattern, but the dominant one for LRA s187(1)(h) purposes is 23 October 2025 – this is the patient safety disclosure that was immediately labelled "insubordination" and directly triggered the recorded admission of retaliatory motive on 31 October 2025.

2. TO WHOM THEY WERE MADE:

May 2025: Senior Manager Ms Mapule Mashele (formal meeting)

September 2025: Operational Manager, Senior Manager Ms Mashele, HR, and the Union (formal written complaint)

October 2025: Line Manager Ms Mary-Ann Jones (email, Lisa Mari Head of the DMS strategic team copied)

3. EXACT SUBJECT MATTER:

MAY 2025 – FIRST FORMAL DISCLOSURE:

During a formal meeting with Senior Manager Ms Mashele in May 2025, I disclosed two things simultaneously: (1) my psychiatric condition (Severe Major Depressive Disorder and PTSD) with formal specialist report from Dr Jacques Malan, and (2) derogatory conduct and threats made by my line manager Ms Jones, including the March 2025 threat "This is the second time you are questioning me; do not let it happen again."

This placed the employer on formal notice of both my disability and the hostile management conduct. Ms Mashele acknowledged she would "personally apply for special leave." Instead, the employer imposed the most punitive option – unpaid leave – and in July 2025 deducted 40.47% of my gross salary (R14,811.77) in a single pay cycle, in breach of BCEA s34(2)'s 25% statutory cap.

This was occupational detriment directly linked to the May disclosure.

SEPTEMBER 2025 – IDENTITY DISCRIMINATION AND OPERATIONAL SUPPRESSION:

Around the 17 September 2025, I presented a Filtering Strategy to improve operational efficiency in managing clinical case files. The strategy was positively received by the team. Immediately after, on the

23 September 2025 Ms Jones publicly misgendered me as "Wilma" in a team-wide Microsoft Teams channel and, in the same message, banned the filtering system with "NO FILTERING" highlighted in red.

This was not a typographical error. It was a deliberate act of identity-based humiliation directly linked to a professional initiative. The employer's senior manager later dismissed it as a "typo" and claimed we had "never had any previous problem" — directly contradicting the May 2025 formal report of Ms Jones's conduct, the unlawful July deduction, and the documented pattern of hostility.

On 29 September 2025 — the same day I filed a formal discrimination complaint — Ms Jones loaded a double punitive unpaid leave entry to my payroll. This was later reversed by Ms Mashele on 1 October 2025, constituting written evidence that the entry was discretionary, retaliatory, and improper.

OCTOBER 2025 — PATIENT SAFETY DISCLOSURE (DOMINANT FOR LITIGATION):

On 23 October 2025, I disclosed via email to Ms Jones that the Gexus and DMS clinical data platforms were not integrated, causing automated false non-adherence letters to be sent to GEMS members (chronic patients) who were in fact adherent to treatment. This is a systemic patient safety risk and scheme administrator breach of Medical Schemes Act obligations.

Ms Jones instructed me to call the affected 75-year-old patient to "gently probe" adherence despite the known system error. I refused on ethical and professional grounds — calling a patient to question adherence when you know the system data is false is professional misconduct. My refusal was immediately labelled "insubordination."

Eight days later, on 31 October 2025, Ms Jones stated in a recorded meeting: "You made a case of harassment against me so I can make a case of harassment against you." This recorded admission establishes direct causal nexus between protected disclosures and retaliatory intent.

THE PATTERN:

May disclosure → 40.47% illegal deduction in July → September disclosure → public misgendering + retaliatory deduction → October disclosure → "insubordination" label + recorded threat → December defamatory CCMA affidavit → constructive dismissal 12 December 2025.

Each disclosure triggered escalating occupational detriment. The October patient safety disclosure is the strongest single thread for litigation because it has the recorded admission, the "insubordination" reframing, and regulatory corroboration (CMS referred to HPCSA; HPCSA referred to SANC; both confirmed it involves professional misconduct). But the May and September disclosures establish the pre-existing pattern that makes the employer's defence untenable.

Marco you have asked (in short). However, this case has complexity, a pattern of retaliation and i do not want to under or over state anything.

Attached is snippets of the recoding between me and Ms Jones.

Yours faithfully,

Wilbur Mathee
Specialist Nurse
PG Dip Health Services Management
BTech General Nursing (Psychiatry, Midwifery & Community Health)

Leadership Roles:Acting Chair – Employee Health & Safety CommitteeMember – Postgraduate Student Quality Committee

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5 attachments

-  **image001.png**
11 KB
-  **image002.png**
328 B
-  **Screen_Recording_20251231_224523_Gallery.mp4**
5.9 MB
-  **Screen_Recording_20251231_224117_Gallery.mp4**
4.2 MB
-  **Screen_Recording_20251231_224948_Gallery.mp4**
12.6 MB

Marco van der Walt <marco@schroterlaw.co.za>
To: Wilbur William Matthee <wilburmat@gmail.com>

Thu, 26 Feb 2026 at 14:20

Hi Wilbur,

Thank you for the below.

For further clarity, and in respect of the October 2025 disclosure, does it only relate to the single incident where the one non-adherence letter was sent out, and was this situation subsequently rectified (i.e. could the client then claim their chronic medications)?

[Quoted text hidden]